

Cavendish Wing
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DISCHARGE SUMMARY

Patient name:	Eveline Achterberg	NHS number:	943 476 5919
Date of birth:	14 March 1962 (age 61)	Hospital MRN:	MRN-7724831
Gender:	Female	Ward:	Cavendish 4 East
Address:	14 Linden Court, Manchester M14 5TQ	Bed:	Bay 3, Bed 12
Telephone:	07700 900145	Admission date:	02 February 2024
Email:	eveline.achterberg@example.com	Discharge date:	14 February 2024
Next of kin:	Pieter Achterberg (spouse), 07700 900146	LoS:	12 days

Care team

Consultant in charge: Dr Marcus Holloway, FRCP (GMC: 6184472)
Specialty: General Medicine / Cardiology
Registrar: Dr Priya Iyer, MRCP
Ward sister: Bernadette Aikens, RN
Registered GP: Dr Helena Cartwright, The Whalley Range Surgery, 412 Withington Road, Manchester M16 8GB. Tel: 0161 555 0204

Reason for admission

Mrs Achterberg, a 61-year-old retired primary school teacher, was admitted via the Emergency Department on 02 February 2024 with a 4-day history of progressive exertional dyspnoea, bilateral ankle oedema, and orthopnoea. She had been seen by her GP Dr Cartwright on 30 January and commenced on furosemide 40 mg OD with limited symptomatic improvement.

Past medical history

- Essential hypertension (since 2008)
- Type 2 diabetes mellitus, diet controlled (since 2017)
- Hypothyroidism on levothyroxine replacement
- Right total knee replacement (2019, Wrightington Hospital)
- Previous smoker, 18 pack-year history, stopped 2003

Discharge diagnoses

Primary: Acute decompensated congestive cardiac failure, NYHA Class III (ICD-10: I50.23)
Secondary: Reduced left ventricular ejection fraction (LVEF 35% on echo)
Contributing: Suboptimally controlled hypertension on admission (BP 178/102 mmHg)

Key investigations

Test	Date	Result
NT-proBNP	02 Feb 2024	4,827 pg/mL (markedly raised)
Troponin T (hs)	02 Feb 2024	18 ng/L (within normal range)
Echocardiogram	05 Feb 2024	LVEF 35%, moderate MR, dilated LA
Chest X-ray	02 Feb 2024	Cardiomegaly, bilateral pleural effusions
Renal function	13 Feb 2024	Creatinine 92, eGFR 64, K+ 4.1
HbA1c	06 Feb 2024	58 mmol/mol (7.5%)

Treatment during admission

Mrs Achterberg was initially managed in the Acute Medical Unit before transfer to Cavendish 4 East under the care of Dr Holloway. She received intravenous furosemide (80 mg twice daily for 5 days, then converted to oral) with significant diuresis and symptomatic improvement. Bisoprolol was uptitrated cautiously from 1.25 mg to 5 mg OD. Ramipril was introduced at 2.5 mg OD and uptitrated to 5 mg OD with monitoring of renal function and potassium. Dapagliflozin 10 mg OD was added on 09 February following the heart failure team review.

She was reviewed by the heart failure specialist nurse Karoline Stenberg on 11 February who provided self-management education and a heart failure passport.

Discharge medications

Medication	Dose	Frequency	Indication	Duration
Bisoprolol fumarate	5 mg	OD (morning)	Heart failure	Long term
Ramipril	5 mg	OD (morning)	Heart failure	Long term
Dapagliflozin	10 mg	OD (morning)	Heart failure	Long term
Furosemide	40 mg	OD (morning)	Heart failure	Long term, review at 4 wks
Atorvastatin	40 mg	ON	Cardiovascular prevention	Long term
Levothyroxine	75 mcg	OD (morning)	Hypothyroidism	Continue as before
Metformin MR	1 g	BD	Type 2 diabetes	Continue as before

Follow-up and actions for GP

For Dr Cartwright at The Whalley Range Surgery, please:

1. Review patient in 1 week to check fluid status, BP, and review symptoms.
2. Repeat U&E and creatinine in 2 weeks following ramipril uptitration.
3. Continue diuretic uptitration as required if signs of fluid overload recur. Consider increasing furosemide to 80 mg OD before adding bumetanide.
4. Refer for cardiac rehabilitation programme at Wythenshawe Hospital (referral form already submitted on 13 Feb 2024, reference WHR-CR-2024-0331).
5. Review HbA1c in 3 months given new dapagliflozin therapy.

Outpatient appointments arranged:

- Heart failure clinic with Dr Holloway — 14 March 2024 at 10:30, Cavendish Outpatients
- Repeat echocardiogram — 09 May 2024 at 14:00, Cardiac Investigations
- Heart failure specialist nurse home visit (Karoline Stenberg) — 21 February 2024 between 09:00 and 12:00

Safety netting advice given to patient

Mrs Achterberg has been advised to monitor her weight daily and seek urgent medical review if she gains more than 2 kg in 3 days, develops increasing breathlessness, ankle swelling, or paroxysmal nocturnal dyspnoea. She has been given the heart failure specialist nurse line (0161 555 0188) for non-urgent queries and advised to contact 111 or attend the Emergency Department for urgent concerns.

Discharge summary completed by:

Dr Priya Iyer, MRCP (GMC: 7283904)
On behalf of Dr Marcus Holloway, Consultant Physician
Date: 14 February 2024
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This document contains personally identifiable patient information. Distribution is restricted to the named clinical team and the patient.
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